

HCN-001: Maria Santos — Interview Transcript

Interview Transcript

PROJECT INFORMATION

Project	Home-Care-AI	Stage	Discovery → Stage 1 (Customer Research)
Artifact	2 (Synthetic Interview Data)	Participant ID	HCN-001
Persona	Home Care Nurse	Next Step	Feed into `/summarize-interview` with other HCN interviews

SESSION DETAILS

Date	2026-03-25	Duration	42 min
Interviewer	PM Lead	Setting	Face-to-face, coffee provided
Location	CareBridge agency office, after shift		

PART A — Narrative Transcript

Opening & Warm-Up

Q: Tell me about your role --- what does a typical week look like for you?

I see fourteen patients a week, give or take. Monday to Friday, four or five visits a day across the eastern suburbs. I do clinical assessments, wound care, medication reviews, care plan updates. But honestly, the visits are maybe sixty percent of my week. The other forty is documentation. Writing up notes, updating care plans, chasing referrals. I probably spend two to three hours every evening after I get home typing up what happened during the day.

Q: Two to three hours every evening? Walk me through that.

I get home around six. I have dinner, put the kids to bed, and then around eight I open my laptop and start typing up the day's visits. I try to do it in the car between visits but the app crashes on mobile and the screen is tiny. So I make voice notes on my phone and then transcribe them at night. I'm usually done by ten, ten-thirty.

[Emotional note] Maria said this matter-of-factly, but her jaw tightened. This is clearly a sore point.

Q: What tools does your agency currently use to track patient status?

We have an EMR — they upgraded it last year. It's worse than the old one. More fields, slower interface. The mobile version crashes constantly. I also use WhatsApp to message families and the coordinator. Paper wound charts for some patients because the EMR doesn't handle images well. And honestly, my own memory. I remember things about my patients that aren't in any system.

Block 1: Falls

Q: Walk me through the last time someone you care for had a fall, or you were concerned they might.

Three weeks ago. Arthur — he's eighty-two, diabetes and heart failure. But it wasn't a fall exactly. He was hospitalised for acute decompensation. When I reviewed his file afterwards, I could see the warning signs. His blood pressure had been creeping up over four visits. His ankles were more swollen each time I saw him. He'd missed two medication doses that I know of. But I didn't connect it in the moment because each visit felt like its own thing. The information was scattered across separate notes.

[Probe] What information did you wish you had that you didn't?

A trend line. Anything. If someone had shown me 'Maria, here's Arthur's blood pressure over the last four weeks, and here's his ankle measurement, and here's his medication adherence' — on one screen, side by side — I would have seen it. I knew something was off. I could feel it. But when you're seeing fourteen people a week, you can't hold all the patterns in your head. The system should have caught that, not me.

[Emotional note] Voice dropped. Genuine guilt. This is a wound.

Q: Have you ever been concerned about a fall specifically?

I worry about falls with at least half my patients. But I don't know when they happen unless someone calls me or I see a bruise at the next visit. Mrs. Chen — Sarah's mother — she fell at two in the morning four months ago and nobody knew until seven AM. I visit her once a week. If she falls on a Tuesday and I don't come until Thursday, that's two days of potential injury progression that nobody sees.

Block 2: Medications

Q: Tell me about the last time there was a problem with a medication.

Arthur again. He was on a diuretic and his potassium supplement. The diuretic dose was increased by his cardiologist, but the potassium wasn't adjusted. I noticed his potassium level was low at my next visit because I happened to check. But the dose change was buried in a hospital discharge summary that I didn't see until three days after it was issued. Nobody flagged it.

[Probe] What's the most frustrating part of keeping track of medications across fourteen patients?

The reconciliation. Every patient has a different GP, a different specialist, a different pharmacy. Changes happen and I find out last. I'm supposed to be the one coordinating, but I'm the last person in the chain to know when something changes. I spend thirty minutes per patient per month just reconciling medication lists. That's seven hours a month on medication admin.

Block 3: Communication & Escalation

Q: When something concerning happens with a patient, how does that information get to you?

It doesn't, usually. I find things out when I walk through the door. Family members sometimes text me on WhatsApp — Sarah Chen texts me constantly — but that's informal. There's no system that says 'Maria, something happened with your patient since your last visit.' I'm always walking in blind.

Q: You said Sarah texts you constantly. Tell me more about that.

Sarah is lovely but she was sending me screenshots of her mother's step count and sleep data from a wristband app. Every week: 'Her steps are down forty percent, is something wrong?' 'She slept less than four hours Tuesday.' Each one added ten or fifteen minutes to my day because I'd have to look at the data, compare it with what I observed, and then respond carefully so I didn't escalate Sarah's anxiety. The data wasn't wrong, it just had no clinical context. A step count drop could mean her mother's mobility is declining or it could mean it rained for three days. I eventually asked Sarah to trust the clinical team to flag concerns. She took it badly. I felt terrible about it.

[Emotional note] Maria paused for several seconds here. Clearly conflicted --- empathises with Sarah but felt overwhelmed.

Block 4: Tools & Workarounds

Q: What tools do you currently use to monitor or coordinate care?

The EMR for official documentation. WhatsApp for quick messages. My phone's voice recorder for notes between visits. A paper notebook for wound measurements because the EMR doesn't support photos well. Google Maps for routing — the scheduling system doesn't optimise drive times. And my memory for everything the system doesn't capture.

Q: Have you tried anything new in the last year?

The agency tried a new care plan template six months ago. It was supposed to make care plans faster. It actually made them slower because it had more mandatory fields. I now spend thirty to forty-five minutes per care plan, starting from a blank template every time. There's no intelligence — no 'based on this assessment, here are recommended goals.' Just blank fields and my brain. I spend more time writing about care than delivering it. By the time I sit down to type up today's visits, I've already forgotten half the details. I'm reconstructing, not documenting.

Block 5: Willingness to Prioritize

Q: How much time per week do you estimate you spend on coordination and communication?

Fifteen to eighteen hours a week on documentation, coordination, and admin. That's on top of the twenty-five hours of actual patient visits. My contract says thirty-eight hours. I work closer to fifty.

Q: If a tool solved the pattern-recognition problem you described with Arthur, what would a fair price look like?

I'd kill for something that just told me: 'Here's what's changed since your last visit. Here's what to watch for today.' Five sentences. That's all I need before I walk through the door. Fair price? I don't care about price — the agency pays for tools. What I care about is whether it actually works on my phone between visits without crashing. Every system we've tried was designed for someone sitting at a desk, not someone standing in a patient's kitchen.

Wrap-Up

Q: Is there anything I didn't ask about that you think is important?

Time. Nobody asks about time. Every new system, every new form, every new process takes more time. I don't need more features. I need my time back. If your system adds thirty seconds to every visit, that's four minutes a day, twenty minutes a week, an hour and a half a month. That's a patient visit I'm losing to admin. Whatever you build, make it faster than what I do now, not slower.

Q: Who else should I talk to?

Priya Sharma — she's our team lead. She sees the systemic stuff I can't. And talk to the patients themselves. Mrs. Chen won't tell you what she tells me, and she definitely won't tell Sarah what she tells me. There's a whole layer of what patients hide that nobody is designing for.

PART B — Filled Note Template

Participant ID	HCN-001
Persona	HCN
Date	2026-03-25
Duration (min)	42
Interviewer(s)	PM Lead
KEY JOBS	Deliver quality clinical care to 14 patients weekly. Accurately document every visit. Reconcile medications across multiple providers. Identify deterioration patterns before they become emergencies. Coordinate between families, GPs, specialists, and the agency.

CURRENT SOLUTION	Agency EMR (crashes on mobile, slow). WhatsApp for family/coordinator messages. Voice recorder on phone for between-visit notes. Paper notebook for wound measurements. Google Maps for routing. Memory for patient context the system doesn't capture. "I spend more time writing about care than delivering it. By the time I sit down to type up today's visits, I've already forgotten half the details. I'm reconstructing, not documenting." "Every care plan I write starts from scratch. No starting point, no suggestion, nothing. Just me and a blank page."
WILLINGNESS	15-18 hrs/week on documentation + coordination (unpaid overtime). Agency pays for tools. Previously rejected: new care plan template (slower, more fields). Key criterion: must work on mobile between visits without crashing. Price is not the barrier — usability is.
EMOTIONAL INTENSITY	Genuine guilt about Arthur's hospitalisation — voice dropped, long pause. Jaw tightened when discussing documentation burden. Conflicted about pushing back on Sarah Chen's data requests. Frustration at management for rejecting Priya's risk review proposal.
ESCALATION BEHAVIOR	Finds out about problems when she walks through the door — no proactive alerting. Relies on intuition and visual observation, not data trends. Information chain: patient/family → WhatsApp → Maria → GP (if she decides to escalate). Weakest link: time delay between events and Maria's next visit.
SURPRISE FINDING	Sarah Chen's 'data parent' behaviour — health data without clinical context created 10-15 min extra work per visit. Maria diplomatically pushed back and felt terrible about it. Raw data to families creates professional friction, not better outcomes.
TOOL MENTIONS	Agency EMR (unnamed, crashes on mobile). WhatsApp. Google Maps. Phone voice recorder. Paper wound charts. New care plan template (failed, 6 mo ago).
REFERRAL	Priya Sharma (team lead, systemic view). Patients themselves — 'there's a whole layer of what patients hide that nobody is designing for.'
FOLLOW-UP ACTIONS	[] Interview Priya Sharma (HCN-003) [] Investigate mobile EMR usability as adoption barrier [] Quantify documentation time across agency nurses